SWACHH BHARAT MISSION HAS ENABLED DISEASE CONTROL BY REDUCING OPEN DEFECATION AND PROMOTING SANITARY HABITS



A toilet in Korali village in Haryana, built under the Swachh Bharat mission

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component to become NHM. It provided the platform for delivery of maternal and child health services to many previously underserved areas and integrated other health programmes on the community connected platform. A major innovation was the training and deployment of locally recruited women as accredited social health activists (ASHA). Celebrated as ASHA workers, a million of them are in the forefront of health service delivery in primary care. NHM has been a key driver of decline in maternal and infant mortality since 2005. ASHAs have been recognised with the Global Health Leaders Award-2022 at the World Health Assembly. ASHAs mobilise pregnant women for institutional deliveries, support immunisation programmes, provide iron and folic acid tablets, contraceptive pills and medicines for tuberculosis. They are now being engaged in the detection and control of non-communicable diseases as well as mental health disorders. Several schemes have been launched within the ambit of NHM to improve health services. These include the Janani Suraksha Yojana (JSY), Janani Shishu Suraksha Karyakram (JSSK) and Pradhan Mantri Surakshit Matrutva Abhiyan (PMSMA).

↘AYUSHMAN BHARAT: Launched in 2018, this has been a major leap in unifying health programmes fragmented by disease-specific approaches and segmented by age groups. Adopting an approach of 'comprehensive primary healthcare' (CPHC), the health and wellness centres (HWCs) cover a wide range of common health conditions. The Pradhan Mantri Jan Arogya Yojana (PMJAY) provides financial protection for hospitalised care to 40 per cent of the population vulnerable to financial shocks. The Health Infrastructure Mission, launched in 2021, and Digital Health Mission strengthen the Ayushman Bharat platform. Enhanced connectivity between these four programmes and their complementarity with state government schemes will advance the journey to universal health coverage (UHC).

↘ACCESS TO MEDICINES: The switch from product patenting to process patenting in the early 1970s enabled large-scale generic drug production by an energised domestic drug industry. India became the 'pharmacy of the world'. India's export of anti-retroviral drugs to South Africa catalysed a global movement for affordable medicines. Currently, there are over 350 Indian drug manufacturing units endorsed by the European Union and many by US regulators. India is the principal supplier of vaccines for global child immunisation programmes. The prowess of India's pharma sector was also evident during the Covid-19 pandemic, when several vaccines were manufactured in high volume for domestic use and export to 101 countries and UN entities.

↘ADOPTION OF DIGITAL TECHNOLOGIES: Despite a late start, India quickly took up adoption and application of digital technologies. The use of telehealth services has risen rapidly after legal barriers were removed. The CoWIN app was remarkably successful in supporting the large countrywide Covid-19 immunisation programme. Point-of-care diagnostic systems are transforming primary care. Technology-enabled non-physician primary care providers can diagnose and manage hypertension and diabetes, providing continuity of care. Supply chain management is more efficient. Disease surveillance has become real-time. The Ayushman Bharat health account (ABHA) number provides a unique ID to individuals, while a unified health interface (UHI) will connect healthcare providers and create digital highways that will integrate health services across the country and provide portability for medical records.

↘DIVERSE HEALTH SYSTEMS: Partly due to conviction and partly for convenience, India has decided to adopt medical pluralism for healthcare. While high quality doctors, nurses and allied health professionals were produced by modern systems, traditional systems of medicine, too, have contributed to the healthcare of many Indians. Shortages of doctors and nurses, aggravated by migration to other countries, were partially compensated by practitioners of other systems. Over time, the health-promoting strength of yoga and the holistic approach of Ayurveda are increasingly being appreciated by other countries. India has a great opportunity to use her strengths across several healthcare and healing systems to develop models of integrative medicine that will benefit the world. ■